WELCOME TO

ELISENCE

Health, wellbeing and human connection



11 November 2026 | Pavilion 4 | ALPHA 8 | Booth A8-01

Care does not end at discharge. Life continues.

The hospital encounter may end while recovery, treatment and everyday life continue.

HEALTHCARE TEAMS

Maintain continuity

How can care teams coordinate follow-up and share authorised information with the next clinician?

PEOPLE & FAMILIES

Navigate recovery

How can people manage medication changes and appointments across fragmented apps and records?

DAILY LIFE

Keep context visible

Nutrition, symptoms, measurements, wellbeing and family support remain part of recovery.

What is ELISENCE?

Our AI-powered ecosystem connects care with everyday life through health, wellbeing and human connection.

WE HAVE

Core technology

In-house technology spanning health, wellbeing, social connection and intelligent services, with implemented components and demonstrated user workflows.

WE CAN

A scoped workflow

Choose one care, wellbeing or research problem and agree how to measure it.

WE BUILD

Agreed integrations

Develop service connections and partner configurations in-house.

> Physical-device proof: food-photo recognition, candidate review, acceptance, save and reload.

The 22-domain ecosystem

A modular ecosystem connecting personal wellbeing, healthcare, intelligent services and institutional opportunities.

Human connection	Everyday health	Care & intelligence	Institutional trust
Social connection	Women and Women+	Disease & stroke risk	Institutional & public services
Voice calls	Diabetes	Medical tracking	Identity, consent, security & audit
Video consultations	Blood & measurements	GP & care access	
Health education	Weight management	Appointments	
ELISENCE Universe	Body Scan	Prescriptions & medication	
	Nutrition & hydration	Medical Intelligence	
	Sensors & devices	Consultation transcription	
		ELISA	

Women and Women+ are distinct in-house programmes with substantial implemented and tested functionality. Application integration is in progress. Service maturity varies across the ecosystem.

Wellbeing is social

People rely on relationships and community to sustain daily routines and navigate care.

EVERYDAY CONNECTION

Keep people close

Profiles, messaging, Pulse and Waves give social connection a place in the ecosystem.

PERSONAL CHOICE

Choose what to share

A person decides whom to involve. Social visibility and access to health records follow separate rules.

A health history that follows daily life

Dated information helps people see changes over time and prepare for the next care visit.

DAILY ROUTINES

Food and weight

Record nutrition, hydration and weight alongside personal notes and routines.

OBSERVATIONS

Measurements over time

Organise symptoms and relevant measurements, including diabetes and blood observations.

CARE CONTEXT

Medication changes

Bring medication references and questions together with relevant, authorised observations for review.

Device inputs depend on the supported workflow. Clinical interpretation needs separate validation.

From discharge to continued care

Illustrative institutional journey. Agree the handoffs and validate them with the provider.

01 Hospital	03 Home	05 Daily health monitoring	07 Specialist review
02 Discharge	04 Medication management	06 Follow-up appointment	08 Continued care

Demonstrated example: meal-photo recognition, review, acceptance, save and reload.

Proposed workflow: coordinate follow-up and share agreed patient-reported context for review.

> Qualified professionals retain diagnosis, prescribing and treatment decisions.

Meet ELISA

Use everyday language to understand supported information and prepare for follow-up after discharge.

ELISA

“Help me prepare for my follow-up.”

ILLUSTRATIVE CONVERSATION

ASK

Explain supported information in plain language.

USE PERMITTED CONTEXT

An authorised appointment status or selected dated entries could inform the response.

SUPPORT THE NEXT STEP

Prepare medication or symptom questions.

Institutional workflows can use authorised context once validated for that service.

How ELISENCE connects services

Conceptual Data Flow | Separate services exchange information through authorised interfaces.

01

Source

Person, supported device or approved partner

→

02

Record owner

Owns the record and retains source, date and identity

→

03

Permission

Checks identity, role, purpose and current grant

→

04

Response

Releases permitted fields and logs the decision

> An access request alone grants no access. Withdrawal must be enforced on later reads.

Privacy, trust & responsible AI

Design each workflow around controlled access, minimal AI inputs and accountable use.

ACCESS

Scoped access

Set the recipient, purpose and permitted fields for each grant.

PROCESSING

Minimise AI inputs

Agree data scope, processors, processing locations, retention and oversight for each use.

EVIDENCE

Assurance

Scoped-access controls have passed isolated testing. End-to-end assurance is assessed for each intended institutional deployment.

> Clinical use requires validation and applicable regulatory assessment.

Post-discharge continuity

Start with one discharge pathway, a named clinical owner and agreed patient-reported information.

SCOPE

Test the handoff

Start with synthetic cases. Agree which observations patients can share and who can review them.

MEASURE

Compare with baseline

Track follow-up completion, preparation time and completeness of agreed information.

AGREEMENT

Pilot to procurement

Set acceptance and safety gates before patient use. Consider a licence or service agreement after evaluation.

Universities & research

Can participants understand, use and withdraw a permission in a connected health workflow?

STUDY

Protocol first

Define participants, purpose, fields, measures and the ethics route. Begin with synthetic tasks.

VALIDATION

Independent review

Measure understanding and withdrawal success. Check traceability and reproducibility.

AGREEMENT

Research contract

Agree sponsorship, methods, publication, permitted reuse and responsibilities for results.

Pharmaceutical & life sciences

Explore a voluntary patient-support programme with approved educational content.

PROGRAMME

Patient education

Provide approved, versioned content and help people prepare questions.

MEASURE

Comprehension

Agree measures for voluntary enrolment, content completion and understanding.

GOVERNANCE

Safety reporting

Set content approval, limits on reported data and adverse-event responsibilities where applicable.

A service agreement can scope delivery. Research requires a separate protocol.

Governments, insurers & public health

Evaluate how defined services can coordinate follow-up and make better use of resources.

PUBLIC ORGANISATIONS

Care coordination

Pilot a referral or follow-up pathway. Measure completion, time to reach the right service and repeated information requests.

INSURERS

Member support

Evaluate voluntary uptake and completion. Measure whether guided follow-up reduces duplicate steps.

> Agree legal authority, data boundaries and procurement before deployment.

European innovation partnerships

ELISENCE can contribute in-house engineering and synthetic workflows for independent evaluation.

EIT HEALTH

Assess fit

Assess published health innovation opportunities against the proposed project and partner roles.

HORIZON EUROPE

Find a call

Identify a relevant call, academic lead, clinical site and governance partners.

CONSORTIA

Scope the work

Propose work on accessible permissions, authorised data exchange or reproducible study outputs.

> These are routes to explore. Call-specific eligibility and funding require confirmation.

Connect care with life

Bring one continuity-of-care challenge or research question.

01

Discuss a paid pilot

Agree one workflow and its success measures.

02

Explore licensing

Define the service, integration and operating responsibilities.

03

Plan a study

Identify an evaluation lead and a shared question.

www.elisence.com **contact@elisence.com**

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